Mr. Jamal Ahmad

Age / Sex : 59 YRS / M

Referred by : SHRI SHYAM POLYCLINIC AND HOSPITAL

Reg. no. : 6999

6999

Registered on : 21/10/2026 02:42 PM

Collected on : 21/10/2026 01:00 PM

Received on : 21/10/2026 01:30 PM

Reported on : 21/10/2026 02:42 PM

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HAEMATOLOGY

COMPLETE BLOOD COUNT (CBC)

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN		13.6	g/dl	13 - 17
TOTAL LEUKOCYTE COUNT	**H**	**12,000**	**cumm**	**4,800 - 10,800**
DIFFERENTIAL LEUCOCYTE COUNT				
NEUTROPHILS		60	%	40 - 80
LYMPHOCYTE		33	%	20 - 40
EOSINOPHILS		05	%	1 - 6
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	< 2
PLATELET COUNT	**L**	**1.00**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT	**L**	**4.08**	**million/cumm**	**4.5 - 5.5**
HEMATOCRIT VALUE, HCT	**L**	**35.5**	**%**	**40 - 50**
MEAN CORPUSCULAR VOLUME, MCV		87.0	fL	83 - 101
MEAN CELL HAEMOGLOBIN, MCH	**H**	**33.3**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**38.3**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV	**H**	**14.8**	**fL**	**6.5 - 12**
R.D.W. - SD		41.1	fL	39 - 46
R.D.W. - CV		13.7	%	11.6 - 14
P-LCR	**H**	**58.9**		**19.7 - 42.4**
P.D.W.	**H**	**17.1**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters:

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

TEST	VALUE	UNIT	REFERENCE
BLOOD GROUP & RH.			
ABO	A		
RH (ANTI -D)	POSITIVE		

SEROLOGY & IMMUNOLOGY

VIRAL MARKER

TEST	VALUE	UNIT	REFERENCE
HIV (CARD TEST)			
HIV - 1	NEGATIVE		
HIV - 2	NEGATIVE		
HEPATITIS C VIRUS, HCV	NEGATIVE		
HBSAG	NEGATIVE		

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation
Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation
A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

BIOCHEMISTRY

LIVER FUNCTION TEST (LFT)

TEST		VALUE	UNIT	REFERENCE
SERUM BILIRUBIN (TOTAL)	H	3.87	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)	H	0.54	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)	H	3.33	mg/dl	0.2 - 1
SGOT (AST)	H	301.57	U/l	0 - 37
SGPT (ALT)	H	235.45	U/l	13 - 40
SGOT/SGPT Method: Calculated		1.28		
SERUM ALKALINE PHOSPHATASE	H	330.6	U/l	44 - 147
SERUM PROTEIN	L	6.01	g/dl	6.4 - 8.3
SERUM ALBUMIN	L	3.44	g/dl	3.5 - 5.2
GLOBULIN		2.57	g/dl	1.8 - 3.6
A/G RATIO		1.34		1.1 - 2.1

LFT Interpretation

Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

Test Significance

Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.

Increased in: Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

TEST	VALUE	UNIT	REFERENCE
SERUM CREATININE	1.40	mg/dl	0.72 - 1.43

Creatinine is a waste product produced by muscles from the breakdown of a compound called creatine. It is removed from the body by the kidneys, which filter almost all of it from the blood and release it into the urine.
Creatine is part of the cycle that produces energy needed to contract muscles. Both creatine and creatinine are produced by the body at a relatively constant rate. Since almost all creatinine is filtered from the blood by the kidneys and released into the urine, blood levels are usually a good indicator of how well the kidneys are working.

Higher than normal level may be due to:
- Blockage in the urinary tract
- Kidney problems, such as kidney damage or failure, infection, or reduced blood flow
- Loss of body fluid (dehydration)
- Muscle problems, such as breakdown of muscle fibers
- Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)

Lower than normal level may be due to:
- Myasthenia Gravis
- Muscular dystrophy.

Low serum creatinine values are rare. They almost always reflect low muscle mass.

SERUM UREA	48.96	mg/dl	18 - 55

RANDOM BLOOD SUGAR	H	160.8	mg/dl	70 - 140

~~~ End of report ~~~

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